

# Rehabilitation and Functioning in the 2030 Agenda

Functioning is the missing dimension of the 2030 Agenda's measurement architecture — and rehabilitation is the health-system intervention most directly concerned with it.

*"Health is a state of complete physical, mental and social well-being and not merely the absence of disease or infirmity."*

— Constitution of the World Health Organization

## THE ARGUMENT IN BRIEF

The 2030 Agenda is not only a set of goals but a governance architecture: what it measures determines what is financed, prioritised, and held to account. Within it, health progress is judged almost entirely through mortality and morbidity — whether people survive, and with what burden of disease — while functioning, whether people can actually live and participate, goes uncaptured. Rehabilitation's contribution runs across the Agenda yet remains invisible in the indicators that steer investment. This brief sets out why functioning should be recognised as a third dimension of population health, alongside mortality and morbidity, and the specific changes needed within the 2030 Agenda and the framework that succeeds it.

## BACKGROUND

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The 2030 Agenda for Sustainable Development is not only a set of goals — it is a governance architecture. The Sustainable Development Goals (SDGs) and their associated monitoring frameworks shape how governments define progress, how international institutions allocate investment, and how countries are held to account across health, education, economic participation, and social inclusion and many other areas. What is measured within this architecture determines what is prioritised, what is financed, and what remains invisible and underfunded.

Within this architecture, health progress is assessed primarily through mortality and morbidity. These have become the dominant lenses through which development is evaluated — underpinning Sustainable Development Goal 3 monitoring, Universal Health Coverage (UHC) reporting, and the financing frameworks through which governments, bilateral donors and development banks direct resources. They measure whether people survive and with what burden of disease. They do not measure whether people are able to live well.

This is a structural gap with consequences that extend across the entire 2030 Agenda. Millions of people survive stroke, road traffic injury, cancer, complications of childbirth, and live with chronic conditions, armed conflict, displacement, and humanitarian crises — yet experience persistent limitations in mobility, cognition, communication, self-care, and participation in daily life. These functional limitations determine whether people can return to work, access education, rebuild their lives after crisis, and participate in society. Yet they are not captured in the indicators through which progress is tracked and reported.

Functioning — the ability to carry out activities and participate in society in interaction with the environment — is the missing dimension. As affirmed in the Constitution of the World Health Organization (WHO), health is not merely the absence of disease or infirmity. Functioning captures whether health gains translate into lives that people can live fully along the life course and into older age. Functioning — what people are able to do and be in their daily lives — should be recognised alongside mortality and morbidity as a third dimension of population health, and is the dimension most directly influenced by rehabilitation, a set of interventions designed to optimise functioning and reduce disability in individuals with health conditions in interaction with their environment. In older age, functioning is closely related to functional ability — the concept WHO's Decade of Healthy Ageing uses for the health-related attributes that enable people to be and to do what they have reason to value. Yet functioning remains absent from the 2030 Agenda monitoring framework, and rehabilitation is not reflected in the principal indicators through which health and development progress is tracked.

The result is a clear and addressable disconnect between political commitment and measurement reality. Member States have committed, through resolution WHA76.6 (2023), to strengthening rehabilitation within health systems. The UHC Political Declarations recognise rehabilitation as a critical component of UHC. The 2030 Agenda commits to leaving no one behind. But without rehabilitation and functioning in the monitoring architecture, neither commitment can be verified, financed, or held to account. This brief sets out what is missing, why it matters, and what needs to change.

## REHABILITATION AND SDG 3: GOOD HEALTH AND WELL-BEING

SDG 3 — Good Health and Well-being — is the primary policy entry point for rehabilitation within the 2030 Agenda. Most of its targets are framed and monitored in terms of mortality and disease incidence: whether people survive, and whether disease is prevented. Within each of these areas, rehabilitation is central to people's functioning, yet because each target is measured by its mortality and disease component alone, that contribution goes uncaptured. Its defining contribution is to the other half of the Goal — well-being — and specifically to functioning: ensuring that, for the people who survive these conditions or live with them, survival and disease control translate into recovered function, participation, and a life that can be lived fully. The entries below describe that contribution for each target — what rehabilitation does for survivors and for those living with each condition.

### 3.1 • Reduce Maternal Mortality

While rehabilitation does not reduce maternal mortality, it is essential for the many women who survive serious maternal complications and live with long-term physical and functional impairments. Rehabilitation following complicated delivery or obstetric injury supports functional recovery and a mother's capacity to care for herself and her newborn.<sup>11</sup>

### 3.2 • End All Preventable Deaths Under 5 Years

While rehabilitation does not reduce child mortality, children who survive premature birth, neonatal infections, or traumatic injury are at high risk of long-term functional impairments. Early rehabilitation maximises functional recovery, prevents secondary complications, and supports developmental milestones.<sup>12</sup>

### 3.3 • Fight Communicable Diseases

While rehabilitation neither prevents nor treats the infections themselves, many communicable diseases — including polio, tuberculosis, leprosy, pneumonia, influenza, Respiratory Syncytial Virus, and post-COVID conditions — result in long-term physical, cognitive, or psychosocial impairments. Rehabilitation restores functioning after acute illness and reduces the long-term impact of these conditions on individuals and communities.<sup>13</sup>

### 3.4 • Reduce Mortality from Non-Communicable Diseases and Promote Mental Health

Noncommunicable diseases (NCDs) including stroke, cardiovascular disease, cancer, diabetes, and musculoskeletal and mental health conditions are among the leading causes of functional limitation globally. Rehabilitation is essential for helping individuals with NCDs achieve optimal functioning and maintain their capacity to participate in daily and economic life. As the burden of NCDs continues to rise, rehabilitation will be increasingly essential to maintaining population health, workforce participation, and quality of life.<sup>14</sup>

### 3.5 • Prevent and Treat Substance Use Disorders

Substance use disorders affect physical, cognitive, and psychosocial functioning across multiple life domains. Rehabilitation services — including occupational therapy, psychosocial support, and vocational training — restore life skills, support daily routines, and support functional recovery and sustained participation in daily life.<sup>15</sup>

### 3.6 • Reduce Road Injuries and Deaths

While rehabilitation does not prevent road traffic crashes, it is central to what happens to those who survive them. Crashes injure tens of millions of people each year — with more than 90% of deaths occurring in low- and middle-income countries, disproportionately among working-age populations — and survivors frequently carry lasting limitations in mobility, cognition, self-care, and participation. Rehabilitation is essential to optimising functioning after road injury, reducing long-term care costs, and supporting reintegration into work and community life. Yet SDG 3.6 is monitored by the death rate alone: the injury half, and whether survivors recover, go unmeasured. The Decade of Action for Road Safety 2021–2030 and the Stockholm Declaration name rehabilitation as a core component of post-crash response<sup>16,17</sup>; measuring survivors' functioning is what would hold that commitment to account.

### 3.7 • Universal Access to Sexual and Reproductive Health

Illness, injury, childbirth, and surgery can all affect sexual and reproductive health and functioning. Rehabilitation addresses conditions including pelvic floor dysfunction, obstetric fistula, and post-cancer sexual health, restoring physical, emotional, and relational well-being.<sup>11,18</sup>

### 3.8 • Achieve Universal Health Coverage

Universal health coverage encompasses the full continuum of care — promotion, prevention, treatment, rehabilitation, and palliation. Rehabilitation is an integral part of that continuum: without it, health systems can deliver survival and treatment but not the functioning that allows people to benefit from them and return to daily life. Ensuring that rehabilitation is available, accessible, and adequately financed within UHC is therefore essential to delivering genuinely people-centred care.<sup>19</sup>

### 3.9 • Reduce Illnesses and Deaths from Hazardous Chemicals and Pollution

While rehabilitation does not reduce exposure to environmental hazards, air pollution — ambient, household, and occupational — and chemical exposures damage respiratory functioning and drive chronic respiratory disease, with further neurological and cognitive effects.<sup>20,21,22</sup> Rehabilitation, including pulmonary rehabilitation, optimises functioning in those living with the resulting impairments and reduces their long-term impact on daily and economic life.

## REHABILITATION AND THE BROADER 2030 AGENDA

The cross-cutting nature of functioning means rehabilitation's contribution extends well beyond SDG 3. By optimising functioning and reducing disability, rehabilitation supports progress across several other SDGs and underpins the “leave no one behind” principle.

| SDG                                                | THE CHALLENGE                                                                                                                                                                                                                                                                                                                                 | REHABILITATION'S CONTRIBUTION                                                                                                                                                                                                                                        |
|----------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>SDG 1 • No Poverty</b>                          | Health conditions that affect functioning create financial strain — out-of-pocket expenditure, catastrophic health spending, reliance on informal support. These pressures compound and increase the risk of falling into or remaining in poverty.                                                                                            | Rehabilitation optimises functioning and supports greater independence, reducing the need for ongoing care. It strengthens ability to sustain livelihoods and mitigates economic vulnerability. <sup>23</sup>                                                        |
| <b>SDG 4 • Quality Education</b>                   | People experiencing functional limitations face challenges accessing and continuing education. Inaccessible environments, lack of assistive technology, and unmet rehabilitation needs lead to school absence, dropout, and exclusion.                                                                                                        | By improving health outcomes, well-being, and autonomy, and through links with inclusive education, rehabilitation contributes to improved access to and participation in education and learning. <sup>24</sup>                                                      |
| <b>SDG 5 • Gender Equality</b>                     | Women and girls face disproportionate barriers to rehabilitation access and carry the majority of caregiving for family members with functional limitations. Unmet needs and caregiving burden constrain education, employment, and participation. <sup>25,26</sup>                                                                           | Rehabilitation enables women and girls to improve their own functioning and independence, and reduces the intensity and duration of caregiving needs — freeing time for education, employment, and community participation.                                          |
| <b>SDG 8 • Decent Work</b>                         | When illness, injury, or noncommunicable disease limits functional capacity, economic consequences extend beyond the health system. Lost functioning reduces workforce participation, productivity, and earnings.                                                                                                                             | Rehabilitation optimises the functional capacity that enables people to return to work, sustain employment, and contribute to economic growth. Its integration into occupational health and social protection systems is essential to SDG 8's targets. <sup>27</sup> |
| <b>SDG 10 • Reduced Inequalities</b>               | People in lower-income countries, older populations, women, persons with disabilities, and those in fragile or conflict-affected settings are most likely to experience functional limitations and least likely to access services.                                                                                                           | Equitable access to rehabilitation is an essential contributor to reducing the structural inequalities that SDG 10 seeks to address. <sup>24</sup>                                                                                                                   |
| <b>SDG 11 • Sustainable Cities and Communities</b> | The environment is a determinant of functioning. Inaccessible, polluted, and poorly planned urban environments restrict mobility and participation and accelerate functional decline, and air pollution drives respiratory and other conditions that erode functioning across the life course, including in children and younger populations. | Rehabilitation, alongside accessible and age-friendly environments, optimises functioning where people live, work, and move — connecting rehabilitation to the age-friendly cities and urban-planning agendas that shape whether people can function. <sup>7</sup>   |

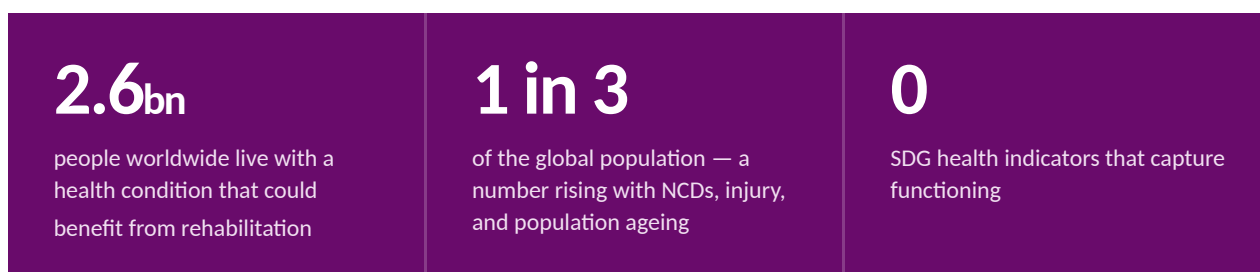
## THE MISSING DIMENSION: FUNCTIONING IN THE 2030 MONITORING FRAMEWORK

Across these goals, functioning is itself the lived health outcome enabling education, economic participation, gender equality, and reduced inequalities. This makes functioning central not only to SDG 3, but to the broader 2030 Agenda. Functioning underpins outcomes across health, work, education, economic growth, and equity, and no single sector or goal owns it. Embedding it in the 2030 Agenda calls for the whole-of-society planning and multisectoral partnership at the heart of SDG 17.

Yet functioning is not systematically measured within global health frameworks. Universal health coverage, a central component of SDG 3, is one of the few places rehabilitation is explicitly named — recognised as part of the care continuum — yet in practice it is not monitored. More broadly, functioning appears in no SDG health indicator, and the health workforce indicator (SDG 3.c.1) does not disaggregate rehabilitation personnel. What is not measured cannot drive investment, prioritisation, or accountability. The measurement infrastructure exists: population-level instruments such as the Washington Group Short Set, WHO Disability Assessment Schedule 2.0 (WHODAS 2.0), and the WHO Model Disability Survey offer tested options aligned to the International Classification of Functioning, Disability and Health (ICF) that can be mapped to a shared set of functioning domains — such as mobility, self-care, cognition, communication, and participation — for population-level reporting. The gap is political, not technical.

This gap will widen as populations age. As the number of older people grows, the dominant health challenge shifts from survival to healthy ageing and the maintenance of functioning — the capacity to move, think, communicate, and take part in daily life that determines whether longer lives are lives worth living. Mortality and morbidity metrics are structurally blind to precisely this shift: they record that people are surviving longer and carrying more disease, but not whether those added years can be lived fully. As the population most affected by the functioning gap becomes the largest, a monitoring architecture that cannot see functioning becomes progressively less able to describe the health of the populations it is meant to serve.

This gap is compounded by structural constraints including insufficient financing, limited workforce capacity, and weak integration of rehabilitation into primary care and health systems. The economic costs associated with reduced functioning are increasingly documented — including impacts on labour force participation, social protection systems, and long-term care — but this evidence base remains limited at the global level.<sup>23</sup> The evidence linking rehabilitation investment to broader social, inclusion, and economic outcomes is at an earlier stage still, representing a critical research and policy gap that the 2030 Agenda's monitoring architecture currently has no mechanism to reveal.



With 2.6 billion people — around one in three globally — living with a health condition that could benefit from rehabilitation, and that number continuing to grow with the rising burden of NCDs, injury, and population ageing, the case for measuring rehabilitation and functioning within global frameworks has never been stronger.

## A CALL TO ACTION

The World Rehabilitation Alliance calls on Member States, global health institutions, and the scientific community to recognise functioning as a third dimension of population health, alongside mortality and morbidity, and to address its absence from the way the Sustainable Development Goals are structured and measured. The SDGs are built around survival and the burden of disease; functioning — the outcome that determines whether those gains translate into lives people can live fully — is the dimension they do not capture, and rehabilitation is the health-system intervention most directly concerned with it.

**Three specific and actionable changes are required:**

- 1 Make rehabilitation visible in the measurement systems that already exist.** The WHO Global Rehabilitation Indicator Set (EB158/26) provides a set of indicators — spanning governance, financing, workforce, information systems, and service delivery — through which countries can strengthen rehabilitation data collection, so that rehabilitation becomes more visible in national health systems. As an immediate and technically achievable step within SDG monitoring, disaggregate rehabilitation personnel within the health workforce density and distribution indicator (SDG 3.c.1). Both carry clear normative backing from resolution WHA76.6 (2023).
- 2 Invest in the evidence base linking rehabilitation and functioning to economic and social outcomes,** so that the contribution of rehabilitation to poverty reduction, employment, education, and inequality can be quantified and held to account within SDG monitoring across all relevant goals.
- 3 Embed functioning in the architecture of the 2030 Agenda and the framework that succeeds it.** Functioning is not confined to SDG 3 — it underpins poverty reduction, education, decent work, gender equality, and reduced inequalities — yet no goal measures it directly. The successor framework will be negotiated for a substantially older world, in which more of the population's health than ever before will turn on maintaining functioning rather than avoiding death; a framework built only on survival and disease burden will be structurally unable to tell whether those added years can be lived, not merely survived.

**Until functioning is systematically measured, the understanding of population health and development progress will remain incomplete.**

*Rehabilitation is essential to health and equitable development, yet its full impact is often underrepresented in the metrics used to guide policy and investment. Ensuring that functioning is systematically measured will strengthen how health systems recognise, finance, and deliver rehabilitation services worldwide.*

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